



PROCEDURE

Lived Experience (Peer) Workforce Reflective Practice

Scope (Staff):	All employees in designated Lived Experience (Peer) roles
Scope (Area):	CAMHS-wide

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

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Aim

To promote high standards of care and safe, consistent Lived Experience (Peer) practice across Child and Adolescent Mental Health Service (CAMHS), and to outline the process for reflective practice and support to Lived Experience (Peer) staff.

Risk

- CAMHS may not meet best practice requirements.
- Inconsistent work practices performed by Lived Experience (Peer) staff.
- Staff wellbeing and retention may be affected.

- Staff professional development needs may not be met.
- Poor client outcomes and experiences.
- Lived Experience (Peer) staff may experience psychosocial harm because of inadequate supports.

Background

All designated CAMHS Lived Experience (Peer) staff require access to reflective practice relevant to their role and experience. This procedure outlines the reflective practice available to staff in designated Lived Experience (Peer) roles.

Additionally, useful information regarding the support strategies available to all CAMHS staff can be found in the Guideline [Support Strategies for CAMHS Staff](#).

Definitions

Peer Worker: In this document, Peer Worker is used as a collective term for staff employed as Peer Workers (first-hand perspective), Carer Peer Workers and Peer Support Coordinators.

Lived Experience (Peer) staff: Any staff member employed in a designated Lived Experience position (including Lived Experience Coordinators and Peer Workers).

Peer drift: When Peer Workers shift towards a medical model of care and deviate from the practices that distinguish them as Peer Workers.

Line Management

The Line Manager for Lived Experience (Peer) staff is responsible for:

- HR matters including leave, mandatory training and supporting employee performance and development
- allocating tasks and overseeing the work of Lived Experience (Peer) staff to ensure they are operating within their role requirements
- oversight of employee Workplace Health, Safety and Wellbeing (WHSW)
- monitoring and ensuring Peer Workers have access to reflective practice with a clinician, as outlined below.

Lived Experience Coordinator

The Lived Experience Coordinator (LEC) is responsible for:

- providing mentoring and practice development for Peer Workers
- ensuring Peer Workers are provided with access to Peer Supervision (in consultation with the Line Manager), including monitoring and coordination of Peer Supervision.
- coordinating the Community of Practice for CAMHS Peer Workers
- provision of training for clinicians who provide reflective practice to Peer Workers around the principles of peer support to bring awareness to the role and prevent peer drift.
- collaborating with Line Managers to oversee the development and implementation of peer support programs throughout CAMHS.

Reflective Practice

Appropriate and adequate reflective practice is a critical factor to the success of Lived Experience (Peer) roles within CAMHS, and safety and wellbeing of the Lived Experience (Peer) workforce. Lived Experience (Peer) staff will have access to the following types of reflective practice:

- *Peer Supervision* with a Lived Experience professional
- *Community of Practice* with other members of the CAMHS Lived Experience (Peer) workforce
- *Reflective Practice* with a CAMHS clinician (for Peer Workers only).

In addition to the above types of reflective practice, Aboriginal staff employed in Lived Experience (Peer) positions will have access to the Cultural Supervision provided to Aboriginal Mental Health Workers, as outlined in [CAMHS Aboriginal Mental Health Service Guideline](#).

Should anyone outside of CAMHS request to join our Community of Practice, the request must be formally considered by Director Specialised CAMHS (Lived Experience Workforce Executive Sponsor).

Confidentiality

For all types of reflective practice detailed below, disclosure of identifying patient information should be avoided wherever possible. As part of reflective practice agreements, both parties agree to keep the content of sessions confidential unless:

- disclosure is authorised or required by law or
- they believe in good faith that disclosure of any information is necessary to protect the safety of any party.

Peer Supervision (Lived Experience Specific Peer Supervision)

Lived Experience (Peer) staff will have access to Peer Supervision. Peer Supervision is discipline specific supervision, provided by a skilled and experienced Lived Experience (Peer) professional who has undertaken Peer Supervision training.

In most instances within CAMHS a LEC will provide Peer Supervision to Peer Workers and Senior Lived Experience (Peer) staff will have access to external Peer Supervision.

Notwithstanding the expertise and experience of the Peer Supervisor, Peer Supervision is a relationship based on a shared experience, where the process is co-created modelling the principles of peer support. It concentrates specifically on the development and maintenance of the Lived Experience practice and skill set to ensure the Peer Worker effectively meets the needs of young people and families/carers. Peer Supervision is trauma informed, strengths based, recovery focused and is a mutual two-way reflection on working practices. It promotes the maintenance of professional and ethical standards.

Peer Supervision allows sharing of collective expertise that aligns with the Lived Experience values and knowledge of the Peer Work sector, to support Lived Experience (Peer) staff with reflective peer practice, prevent peer drift and expand their knowledge base. It can also be a safe space for Lived Experience (Peer) staff to debrief on the challenges of the role.

The table below provides examples of aspects of Lived Experience practice that Peer Supervision can support.

Peer supervision scope examples
Managing tensions: • Operating within systems but not ‘of’ systems • Ethical dilemmas • Lived Experience/Support Worker differences • Boundaries • Maintaining confidentiality • Difficult conversations and scenarios • Co-optation to other practices • Impact of role on lived experience e.g. re-traumatisation, compassion fatigue • Working in isolation
Peer practice reflection: • Avoiding peer drift / staying ‘peer’ (integrity of practice, values and principles, practice frameworks) • Clarity of role • Purposeful application of lived experience knowledge to practice • Responsibilities for own recovery, self-care practice • Debriefing in a safe space • Guiding frameworks: e.g. Trauma Informed Care and Practice, Recovery Approach
Skills development: • Lived experience skills and knowledge development, access to training • Peer and lived experience network opportunities • Reflective practice • Career progression and personal growth

Table Reference: [Mental Health Peer Supervision Framework, 2019 Lived Experience Workforce Program, Mental Health Coalition South Australia \(MHCSA\).](#)

Lived Experience (Peer) staff can access individual Peer Supervision in a format manner that suits them (in person, via telephone or videoconferencing). Peer Supervision should be conducted regularly with a specific time allocated. The session time should be protected and prioritised. It should be conducted in an appropriate, safe and private setting.

To mitigate any conflict of interest or role confusion (in relation to the mentoring role of a LEC), Peer Supervision will be offered where possible by a Peer Supervisor who is not currently working within the same Directorate as the Peer Worker. Where possible, Peer Workers should have the option of receiving Peer Supervision by a Supervisor who shares a similar lived experience (Carer to Carer/ Consumer to Consumer).

Upon commencement to a Lived Experience (Peer) role, Peer Supervision will occur 4 – 6 weekly, for a period of 1 hour, and can be accessed at other times if required. The *Lived Experience Peer Supervision Agreement template* must be completed prior to commencement of sessions.

Community of Practice

Peer Workers will be invited to participate in a Community of Practice with other Peer Workers from all directorates within CAMHS. The Community of Practice includes both professional development and group reflective practice. This will be a space to share information and co-reflect on the lived experience practice between directorates and disciplines.

This will occur at least 4 times per year with additional sessions where required. The Community of Practice will be facilitated by a LEC who has undertaken Peer Supervision training.

External Peer Supervision

External Peer Supervision may become necessary in instances where CAMHS does not have an appropriately trained Peer Supervisor available or where additional learning goals are required. An *Application Form for External Peer Supervision* must be completed.

- Approval from the Head of Service and Director is required prior to the appointment of the external supervisor.
- Approval from the Reform Manager (current cost centre owner) is required prior to the appointment of the external supervisor for any costs involved including allocation to the cost centre.

As with internal Peer Supervision, the *Lived Experience Peer Supervision Agreement template* must be completed prior to commencement of sessions with an external supervisor.

Reflective Practice (with a clinician)

Peer Workers will have access to reflective practice with a CAMHS clinician within the same team, with relevant clinical knowledge and skills who is different from their Line Manager. The CAMHS clinician should have previous experience working alongside Lived Experience (Peer) staff and/or have undertaken education or training to understand the guiding principles for Lived Experience (Peer) workforces. The CAMHS clinician is an ally for the Peer Worker and may undertake internal advocacy to support and champion the role.

The purpose of this reflective practice is to:

- support staff in their professional role, including ongoing learning and skill development
- provide a safe and supportive environment in order to cope with the demands of the clinical environment
- reflect upon and review current practices
- discuss strategies for complex situations and de-briefing on emerging issues
- help staff to consider links between peer support and other parts of the organisational context.

Within this reflective practice the CAMHS clinician should be mindful of:

- reducing peer drift
- the potential for power imbalances and need to create mutual, 2-way learning relationships through the process of purposeful discussion.

This reflective practice should be conducted regularly with a specific time allocated. The session time should be protected and prioritised by both staff. It should be conducted in an appropriate, safe and private setting.

Upon commencement to a Peer Worker role, reflective practice with a clinician will occur 2 - 4 weekly, for a period of 1 hour and may also be accessed at other times, for example as part of critical incident debriefing. After a period of 3 - 6 months the frequency of this reflective practice can be reassessed by both parties to best meet staff needs.

The *Reflective Practice: Clinician and Peer Worker Agreement template* must be completed prior to commencement of reflective practice sessions.

Related CAHS internal policies, procedures and guidelines

[Engaging with Peer Workers Guideline](#)
[Support Strategies for CAMHS Staff](#)
[CAMHS Aboriginal Mental Health Service Guideline](#)

References and related external legislation, policies, and guidelines

[Lived Experience \(Peer\) Workforce Framework Queensland Health 2023](#)
[Mental Health Peer Supervision Framework, Lived Experience Workforce Program and Mental Health Coalition of South Australia 2019](#)
[Lived Experience Workforce Guidelines | National Mental Health Commission](#)

This document can be made available in alternative formats on request.

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